

إِسْمُ الْمَرِيضِ رُبَاعِيًّا : حَلِيمَه مُحَمَّد مُحَمَّد رَمْضَان

المحافظة : القاهرة

الحي / المدينة : عين شمس

العنوان : ---

تاريخ تسجيل الإستمارة : 05-10-2026

رقم البطاقة : 28308010112761 رقم التلفون : 01066438880

العمر (تاريخ الميلاد) : 01-08-1983 المهنة : لا تعمل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطه العلاج : علاج مبادرة سرطان الثدي علاج كيماوي تاكسان (دوسيتاكل) [14152]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

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Name: حليمه محمد محمد رمضان National ID: 28308010112761	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

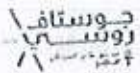
المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Docetaxel chemotherapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



١٥٣٥
ن. عيسى
فيسيو
كاربو بلاتين





مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

السرطان
المرأة
مصر

05/20/2026

MDT FORM

Name: <u>فايزة محمد رمضان</u>	HRN: 9202
Age: 43 Y	ID: 28308010112761
Menopausal Sta: premeno	Phone: 01066438830
NRS SCORE:	Marital Status: Married
BMI:	Family History: paternal aunt query
CO Morbidity: CKD	Reason Of MDT: new case

Clinical Data:

BX:
ER: 8/8 PR: 8/8 HER2: 3+ KI67: 15%

Sonomammography:

Mets Workup:

History:

42 yr old female patient, premenopausal, presenting with left breast lump of four months duration, pt has past medical hx of CKD under dialysis 3x week, granulomatous mastitis of left breast under steroids.

The condition started 4 months ago by left breast mass. BSM revealed rt breast BIRADS 4 lesion and left breast inflammatory process .. biopsy from right breast revealed Infiltrating Duct Ca GII ER PR 8/8 HER2 3+. KI67 15%, while left breast revealed granulomatous mastitis patient started steroid therapy and query immunosuppressant therapy 1.5 m ago, CT CAP and BS revealed query multiple lytic bone lesions, however, patient has highly elevated PTH levels.

by Dr Shalmaa Lasheen

Recommendations:

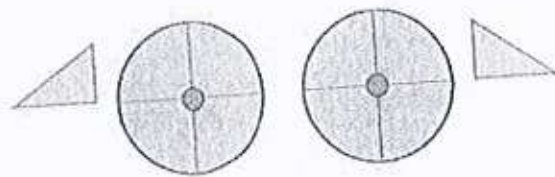
review of bone scan and CT CAP
query bone disease (metabolic vs metastatic)

MDT Outcome & Plan

After reviewing pt's file, pt has CKD and is on dialysis with markedly elevated PTH, bone scan uptake is more likely due to metabolic disease rather than mets, especially that the pt has early stage disease.
Bring mammosonography films for surgical decision.
Meanwhile, pt will be for NACT TCHP x 6 guided by clinical response....re-MDT with mammosonography films.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
<u> </u>	<u> </u>	<u> </u>	<u> </u>	<u> </u>	<u> </u>



T N M

Name: حليمه محمد محمد رمضان			
MR. : 9202			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a premenopausal individual with right-sided invasive ductal carcinoma, clinical stage T2 N1 M0, estrogen-receptor 8/8 positive, progesterone-receptor 8/8 positive, HER2 3+, Ki-67 15%. There are osseous lesions but bone scan is negative. The patient has chronic kidney disease on dialysis for seven years (dialysis on Sunday, Tuesday, and Thursday) with an ejection fraction of 64%. Additionally, there is left granulomatous mastitis, level 1 pathological axillary lymph node involvement, and microcalcifications on the right side. Imaging shows a right breast mass measuring approximately 9.6 × 9 × 7 cm. Multidisciplinary discussion recommended four cycles of Adriamycin (doxorubicin) 95 mg/m² and cyclophosphamide (Endoxan) 950 mg/m² (AC) followed by four cycles of a taxane-based regimen, then right-sided sentinel lymph node surgery. On examination, a 2 × 2 cm mass was noted at the 12 o'clock position. Because the drug Adriamycin is not cleared by dialysis, it can be administered today as cycle 1 of AC with a 20 % dose reduction, with tolerance to be assessed before the next cycle. The patient's weight is 93 kg, height 160 cm, and surface area 2 m². On examination of the chest, there were a few sonorous rhonchi. The patient continued supportive care and received cycle 2 of the AC regimen (Adriamycin 95 mg/m² and Endoxan 950 mg/m²) as neoadjuvant therapy. Clinical pharmacy intervention documented an absolute neutrophil count of 1.1 × 10⁹/L for one dose of Zarzio. To date, the patient has completed two cycles of the AC regimen.			
تحريرا في ,			
		التاريخ :	18-09-2026
		الوقت :	
		الطبيب المعالج :	dina makram
يعتمد ,			



Patient's Name: HALEMA MOHAMED MOHAMED RAMADAN
Date: 2026-04-12
Referred by: Prof.Dr. CT CAP
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported right breast carcinoma,

(I) Chest:

- Left breast lesion 2.2 x 1.5 cm .
- Bilateral enlarged axillary lymph nodes , the largest on the right side measures 1.6 x 1.6 cm and on the left side 1.2x 0.8 cm
- No sizable hilar or mediastinal lymph nodes
- Clear both lung fields .no sizable pulmonary masses or consolidation patches
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous low CT density , no focal lesions
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys, aorta and IVC.



Patient's Name:

HALEMA MOHAMED MOHAMED RAMADAN

Date:

2026-04-12

Referred by:

Prof.Dr. CT CAP

Hospital:

Goustave Roussey International Egypt

- Normal CT appearance of the spleen .
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Dorsal ,lumbar and iliac lytic bony lesion
- No vertebral structural collapse .

OPINION:

- Left breast lesion for sonomammographic correlation .
- Dorsal ,lumbar and iliac lytic bony lesion for correlation with bone scan

Dr.Reham Khedr, MD

Name : Halima Mohammed Mohammed
Ramadan
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 20-04-2026 09:59
Medical No. : 354835
Ins.Company : تعاقد مستشفى

Tc-99m MDP SKELETAL SCINTIGRAPHY

History: 42-year-old female patient with history of breast carcinoma, complains of unexplained bone aches.

Technique: Anterior and posterior whole-body scans 2 – 3 hours post injection of 20 mCi of the tracer.

Findings

- The entire axial and appendicular skeleton are visualized with intensely increased, diffuse radiotracer uptake.
- There is a marked reduction in extra-osseous background activity, resulting in an unusually clear "bone-to-background" contrast.
- There is an **absolute absence of tracer visualization** in the kidneys and the urinary bladder (the "absent kidney" sign).
- The intense uptake is widespread across the skeleton, outcompeting renal excretion for tracer localization.

IMPRESSION:

- The scan findings are suggestive of metastatic super scan rather than metabolic bone disease (hyperparathyroidism/renal osteodystrophy) or hematologic malignancy for clinico-laboratory and radiological correlation.

Much obliged,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient Name	: الأستاذة / حليمه محمد محمد	Request No.	: 13005378
Patient No.	: 28308010112761	Request Date	: 10/03/2026
Path No.	: 653/26	Verification Date	: 14/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مياثره
Age	: 42 Y	Department	: Out Patient
Referring Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical data

Patient with right breast cancer (newly diagnosed as invasive duct carcinoma), Presented with abscess on left side biopsy before was duct ectasia.

U/S results:

-An extensive cloudy highly turbid left breast fluid collection, breast abscess , with surrounding interstitial breast edema. and multiple enlarged inflammatory left axillary lymph nodes , for close follow up.

Nature of specimen

-Multiple biopsies from left breast.

Gross specimen

-Multiple fragmented grey white tissue pieces admixed with blood clots&fat collectively measured 3.5x3.5 cm .The largest piece was 2.5x1 cm, multisectioned and totally submitted.

Microscopic examination

-Examination revealed lobulocentric mixed inflammatory infiltrate composed of lymphocytes, neutrophils and scattered multinucleated giant cells.
-There were round to oval cystic spaces (lipid vacuoles) rimmed by neutrophils (microabscesses).
-There is associated also nonnecrotizing granulomas.

Diagnosis

Left breast abscess, incisional biopsy :
-Cystic neutrophilic granulomatous mastitis.
N.B: Correlate with microbiological evaluation.

Verified by :

Dr.Heba Hasan

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / حليمه محمد محمد
Patient No. : 28308010112761
Path No. : 653/26
Sex : Female
Age : 42 Y
Referreing Doc :

Request No. : 13005378
Request Date : 10/03/2026
Verification Date : 14/03/2026
Financial Category : جوستاف روسي - مبادرة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Signature
Prof. Dr.Maha Guimei
Dr.Heba Hassan Adly

Verified by :
Dr.Heba Hasan

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / حليمه محمد محمد	Request No. : 13004843
Patient No. : 28308010112761	Request Date : 03/02/2026
Path No. : 326/26	Verification Date : 16/02/2026
Sex : Female	Financial Category : جوماتف روسي - مبادره
Age : 42 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data

Right breast mass. Left breast inflammatory process not responding to treatment.

Nature of specimen

1) Right breast mass : Tru cut needle biopsy collected on 3 / 02 / 2026 , at 11:55 am.

Seconds

2) Left breast mass: Tru cut needle biopsy collected on 3 / 02 / 2026 at 12: 05 pm.

Gross examination

1) Right breast mass: Multiple grey white tissue cores ranging in size from 0.7 cm up to 1.3 cm.

2) Left breast mass: Multiple grey white tissue cores ranging in size from 1cm up to 1.2 cm.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: HER2/NEU (4B5), KI67 (30-9), PR (1E2), ER (SP1).

Microscopic examination

1) Right breast mass:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation + nuclear pleomorphism+ mitosis): (2+2+2) Grade 2
- * DCIS: Absent
- * Lobular intra epithelial neoplasia : Absent
- * Angioinvasion: Positive
- * Microcalcification : Absent.
- * Perineural invasion: Absent.
- * IPT (Positive control showed appropriate reactivity) :
- * ER: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
- * PR: Strong nuclear reaction in 90% tumor cells (Allred score 8/8)

Verified by :

Dr.Heba Hasan

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Patient Name : الأستاذة / حليمه محمد محمد
Patient No. : 28308010112761
Path No. : 326/26
Sex : Female
Age : 42 Y
Referring Doc :

Request No. : 13004843
Request Date : 03/02/2026
Verification Date : 16/02/2026
Financial Category : جوستاف روسي - مبادره
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

* HER2: Strong complete membrane reaction in >10 % of tumor cells (3+)
* Ki 67: 15 %

2) Left breast mass:

Examination of the slides revealed normal breast tissue showing irregularly dilated ducts with foamy histiocytes within the luminal secretions and infiltrating the wall of the involved ducts. Surrounding stroma shows infiltration by lymphocytes, plasma cells, neutrophils and histiocytes. No evidence of atypia or malignancy.

Diagnosis

1) Right breast mass, Tru cut needle biopsy
- Invasive duct carcinoma Grade 2 (tubular differentiation 2, pleomorphism 2, mitosis 2)
(according to Nottingham grading system of breast cancer) ,
IPT:

* ER: Positive (score 8/8).
* PR: Positive (score 8/8).
* HER2: Positive (score +3)
* Ki67: 15 %

2) Left breast mass, tru cut biopsy :

- Consistent with Duct ectasia.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 0+: as defined as incomplete membrane reaction in less than 10 % of tumor cells. IHC 1+: as defined by incomplete membrane

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Dr. Heba Hasan

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Sex	: Female	Financial Category	: جوستاف رومبي - ميانره
Age	: 42 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

staining that is faint/barely perceptible and within $< 10\%$ of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within $< 10\%$ of the invasive tumor cells or complete and circumferential membrane staining that is intense and within $< 10\%$ of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense $< 10\%$ of tumor cells.

Signature:

Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :

Dr.Heba Hasan

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Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



Presidential Program for Women's health

Name: Halima Mohamed Mohamed

Date: 25-01-2026

Bilateral Full field Digital Mammography

Clinical summary: 42 year old female with painful left breast from few days

Technique

Mediolateral oblique and CC views

High resolution ultrasound

Detailed Findings

Right breast

- Fairly dense nodular breast parenchyma (ACR b).
- UOQ: spiculated dense lesion harboring micro-calcification with segmental area of calcifications seen anterior to lesion in area about 9x7x8cm.
- Ultrasound demonstrated between 11-12 o'clock shows spiculated hypoechoic solid mass, measuring 20x10mm, away from nipple by 7 cm and skin 6mm with mild internal vascularity on color Doppler study. there dilated retroareolar ducts 3mm seen extending from nipple to ducts ?? Intraductal extensions
- Normal skin/nipple/areolar complex.
- Preserved axillary LNs.

Left breast

- Fairly dense nodular breast parenchyma (ACR b).
- UOQ: focal asymmetry
- NO distortion or suspicious clusters of microcalcifications.
- Ultrasound between 12-4 o'clock shows dilated retroareolar ducts 5mm surrounding by alter of tissue echogenicity, mild interstitial edema and mild skin thickening
- Reactive axillary LNs.

Conclusion:

-right breast (BIRADS 4c) malignant looking solid lesion for biopsy

-left breast (BIRADS 3) ductectasia with inflammatory process for proper management (biopsy is recommended if no response to treatment to exclude cellular activity)

Best regards,
Dr. Marwa M.Hassan

25/01/2026

25/01/2026

Dr. Marwa M.Hassan

25/01/2026